MY DOCTOR
FAMILY PRACTICE

EDITED: 23 March 2026

GENDER-AFFIRMING HEALTHCARE GUIDELINE
Client information for masculinising hormone therapy

Many transgender and gender diverse (TGD) clients choose to seek gender-affirming care in the form of hormone therapy. The decision to commence hormone therapy rests with you, the client, and not with your healthcare provider. The informed consent model of treatment gives you agency over this decision – it is the role of your doctor to support and guide you through this process safely and effectively.

You may have read up, or heard from other TGD persons, about hormone therapy. Some of the information that you may have come across could potentially be out of date or inaccurate.

If you have any questions or concerns at any time, then you should always feel free to raise these concerns with your healthcare provider.

Please remember that every client is unique and will respond differently to medication, and that one client's treatment programme might differ substantially from another's due to a variety of physiological and medical factors. Try not to compare the treatment that you are receiving with that of anyone else – your journey is your own. If you have concerns about the efficacy of your treatment, then make a point to discuss this with your provider at your next appointment.

Before commencing hormone therapy, there are a few points that are worth considering. This informed consent document will draw your attention to some of these points, as well as outlining the expected effects, side effects and risks associated with hormone therapy, to ensure that you have all the information you need to make the best decision about your body and your health.

The role of psychotherapy in TGD clients

Seeing a psychologist is not a requirement for initiating hormone therapy. While previously, some healthcare providers required a letter of diagnosis or referral, this is no longer necessary under international best practice.

Your doctor will ask you some questions to determine that you have a good support structure as you move forward. This is not because taking hormone therapy itself

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necessitates this; rather, as many TGD clients experience, navigating the world is already difficult, and hormone therapy – although often resulting in many positive and beneficial changes – can also lead to some emotional lability. A solid support structure looks different for everyone, but may include friends, family, support groups, therapists and counsellors. Your doctor may suggest or offer you referrals to support groups and therapists, if you indicate that you may benefit from these.

The role of endocrinologists and other specialists

Hormone therapy does not need to be prescribed or monitored by an endocrinologist. Prescribing hormone therapy is well within the realm of a suitably skilled general practitioner (GP) or family physician. Not all GPs, nor all endocrinologists, have experience in managing gender-affirming hormone therapy, and the expertise of the clinician should be the guiding factor in determining who prescribes and monitors your hormone therapy.

You may benefit from seeing other allied health professionals, such as speech-language therapists, or from medical specialists such as plastic surgeons. Not every TGD client will necessarily want to pursue these options, and you should discuss your individual goals with your prescribing doctor.

Home affairs and gender marker (South African Nationals ONLY)

If you are a South African Citizen and wish to update your gender marker on your birth certificate and identity document (ID), then the Department of Home Affairs requires two letters from healthcare professionals which state that you have undergone medical or surgical gender reassignment. Either of these is sufficient; you do not need to have had surgery to update your gender marker.
Unfortunately, at present, gender markers in South Africa are binary; thus, an ID document can reflect either female or male. There is no unspecified or non-binary marker.

Your prescribing doctor can write one of your letters for the Department of Home Affairs and should be able to refer you to another healthcare provider to write the second letter.

If you wish to change your name, then this process should be undertaken separately (either before or after) to the process of updating your gender marker. This does not require supporting letters from healthcare providers.

Potential challenges with legal documents: Some TGD persons have trouble with banking, registration as a student and writing examinations, registration of motor vehicles, etc. while they are awaiting their new documents. Some are accused of potential fraud because they do not look the same as the photo in their ID or their name may be different from the name on their qualification certificates. To change one's gender marker and names will take a while. You may want to consider how you will deal

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with these challenges in the interim. You may also want to consider if you need to change other documentation, for example your matric certificate.

Other important aspects to explore:

Potential impact on emotions: The impact of testosterone can be very diverse and individualised. Your mood may fluctuate. For example, often a TGD man may struggle to cry, and their emotions may become less intense. Some also experience increased irritability.

Potential impact on relationships with family and significant others: Have you thought about the possible impact on your relationship with significant others? If you are in an intimate relationship, this may change when you start on hormones, and relationship roles may need to be re-negotiated. A partner may grieve the loss of aspects of who you were and the way the relationship used to be. What possible impact can it have on your family and how will you be able to deal with your family's response? Have you considered the impact of the change of gender role in your family? Have you considered the impact of potential loss of fertility? Are there children that may be impacted and are the children prepared?

Potential change in sexual orientation: It is possible that your sexual orientation may remain constant or shift, either temporarily or permanently (e.g. shift in attraction or choice of sexual partners, widened spectrum of attraction, shift in sexual orientation identity)

Potential impact on safety: In some settings, the physical changes on hormones may affect your safety, with persons who do not fit into society's expectations of male or female being at increased risk of violence.

Potential impact on employment: Some TGD persons experience discrimination in the workplace, or struggle to obtain employment. This may be more difficult when your legal documents (e.g. ID) and your appearance do not match.

Potential grief and loss: Some TGD persons experience a sense of loss. A TGD man may lose certain gender roles in the family.

Taste changes: Some TGD persons experience a change in their taste sensations, and their likes and dislikes of certain foods.

Body odour change: Some TGD persons experience a change in their body odour on hormone therapy.

Appetite and sleeping patterns: Often the TGD person will experience an increase in appetite. This could cause weight gain. Sleeping patterns may also be affected.

Masculinising hormone therapy

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Masculinising hormone therapy is prescribed for assigned-female-at-birth clients who wish to masculinise. The backbone of masculinising hormone therapy is testosterone therapy. No additional medications are necessary to suppress oestrogen, since testosterone is able to do this alone.

The biggest concern with testosterone therapy is the risk of liver and cardiovascular disease. Testosterone use can adversely affect the **liver**, which is an organ vital to detoxifying the blood, and metabolising medications and dietary nutrients. Testosterone has also been observed to cause an **increase in LDL (commonly known as 'bad') cholesterol**, and a **decrease in HDL (commonly known as 'good') cholesterol.** These changes in metabolic profile can increase a client's risk of heart attacks or strokes to levels similar to those seen in cisgender men.

Comparison of testosterone preparations			
Type	Examples	Advantages	Disadvantages
Short-acting injection	Depo-testosterone	• Accessible at most pharmacies • Affordable • Once-a-week dosing	• Requires knowledge of injection technique • Requires disposables (syringes, needles and alcohol swabs)
Long-acting injection	Nebido	• Accessible at most pharmacies • A single dose lasts approximately 3 months	• Requires knowledge of injection technique • Requires disposables • Achieving the correct dose can be difficult with long dosing intervals
Topical gels or creams		• No injection needed	• Available only from compounding pharmacy • Skin absorption varies between clients

Your doctor will discuss with you the various options for hormone therapy that are available, and help you to decide which form of treatment is best for you.

Additional medications:

For clients who wish to achieve suppression of menstruation, but have not done so on testosterone alone, progesterone may be added.

Some clients on masculinising hormone therapy may experience vaginal dryness and thinning (atrophy) of the vaginal tissues due to reduced estrogen levels. In these cases, a low-dose topical vaginal estrogen can be helpful. This treatment works locally to improve tissue health, reduce discomfort, and prevent symptoms such as dryness, irritation, or pain, without significantly affecting overall hormone goals. It can be a safe and effective option when used under medical guidance.

Some clients will use topical minoxidil in order to achieve fuller facial hair growth.

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Costs of hormone therapy

It's important to remember that not only do different clients have different needs in accessing hormone therapy, but also that prices for medication may vary between different pharmacies, and that these prices may fluctuate over time. The majority of clients can expect to spend R600-R800 per month on hormone therapy. This does not include the cost of monitoring blood tests or doctor's visits.

Masculinising Hormone therapy Cost Guide (edited. April 2026)		
Initial Consultation with baseline Bloods		
	In-person consultation	**Virtual consultation**
Consultation	R450	R350
Baseline blood tests **Hb; Creatinine; ALT; T.chol; HIV; HebBsag; Syphilis; (+ Pregnancy test & Glucose if Virtual consultation)**	+/-R325	+/-R490
Medication	Depo Testosterone **10ml** vial R623.02 (Dischem 23 January 2026) Depo Testosterone **5ml** vial R436.00 (Fagron Compounding pharmacy)	
	Nebido 4ml vial = R2421.50 (Dischem 23 January 2025)	
Follow up Blood tests at 1 and/or 3 and 6 months on treatment		
Monitoring blood tests(Hamadi Labs) **Hb; ALT; Total Testosterone**	+/-R200	
Virtual Consultation to review blood results	R350	

*Pricing subject to change

Changes that occur when using hormone therapy

The changes you will experience on hormone therapy often take some time to develop fully. Some of these changes are reversible and will disappear should you discontinue hormone therapy. Others are irreversible and will persist even if you stop taking your hormones.

The timeline for these changes to begin is variable, but most of them will only reach their maximum degree after 3–5 years on hormone therapy.

Reversible changes

Such changes include:
• gain of muscle mass and increased strength
• changes in body fat distribution, possibly associated with weight gain (increased fat deposition in the abdomen, and decreased fat in breasts, buttocks and thighs)
• coarser and thicker skin

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• increased acne
• coarser and thicker body hair
• increased red blood cell count
• increase in sex drive
• changes in mood and emotional response (often initially an increase in anger and aggression, among other emotions)
• cessation of menses and ovulation
• dryness of the genital tissues.

Irreversible changes

Such changes include:
• hair loss or male pattern balding may occur
• facial hair growth
• deepening of the voice
• enlargement of the clitoris
• infertility

Limitations of hormone therapy

It is important to understand that there are certain features that hormone therapy cannot alter. Among others, these include:
• presence of breast tissue – hormone therapy can reduce fat deposition in the breasts, and make them smaller, but will not result in loss of actual breast tissue
• bone structure – hormone therapy will not change the structure of your pelvis or cause you to grow taller.

Important risks associated with hormone therapy

As with any medication, hormone therapy carries with it certain risks. Some of these risks can be mitigated or reduced by lifestyle factors, while others are independent risks that cannot be altered.

It is important to be fully aware of the risks associated with hormone therapy before making the decision to start your treatment:
• high cholesterol or blood fats
• increased red blood cell count
• high blood pressure.

All of the above can lead to, or worsen cardiovascular disease, or lead to strokes. These conditions can be life threatening.

Psychiatric symptoms including mood disturbances, anxiety or psychosis, especially if there are pre-existing mental health conditions. If you have been diagnosed with a mental health condition and/or use psychiatric medication, then you need to discuss

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the starting of hormone therapy with your doctor. Using hormones can interact with various medications and have an impact on your mental health.

Some of the risks mentioned are modified by other factors. Notably, cardiovascular and clot risk are worse in clients who:
• smoke
• use alcohol
• have pre-existing medical conditions, such as diabetes, high blood pressure and high cholesterol.

Fertility

Although not all clients become infertile on hormone therapy, and some might regain fertility if they stop hormone therapy, many may become irreversibly infertile. Hormone therapy is not a replacement for effective and responsible contraceptive use.

All clients considering starting on hormone therapy should consider using a cryobank to preserve genetic material, in case they wish to conceive genetically related children at a later stage. Even if this is not a priority for you at this stage in your life, please consider the possibility that your perspectives might change with time, and that it is ideal to store material before starting hormone therapy, rather than trying to regain fertility once you are already taking hormones.

Egg Freezing (Oocyte Cryopreservation)

Egg freezing is a medical way to collect and store your eggs now so you can use them later (e.g., with IVF) when you're ready to build a family. It involves hormone stimulation, egg retrieval, and long-term freezing.
Who it's for:

- People planning **testosterone or surgeries** that may affect fertility
- Those who want to **delay parenthood** but preserve biological options
- Anyone wanting reproductive autonomy in the future

How it works:

1. **Consultation** with a fertility specialist
2. **Hormone injections** for about 10–14 days
3. **Egg retrieval** under light sedation
4. **Freezing and storage** of your eggs until use

Fertility Clinics in Cape Town & Nearby Offering Egg Freezing
Cape Town Region
• **Aevitas Fertility Clinic** – Comprehensive fertility care including egg freezing and storage. Offers consultations and tailored planning for fertility preservation.
• **Cape Fertility** – Large fertility clinic offering egg freezing (vitrification) and IVF programs; good option for personalised fertility preservation.
• **HART Fertility Clinic, Cape Town** – Offers egg and embryo freezing as part of full fertility services.

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• **Wijnland Fertility Clinic** – In Stellenbosch near Cape Town with egg freezing and fertility preservation support; offers ova bank and donor programs.
• **Fertility Solutions** – Specialist clinic that can support referrals or initial fertility assessments (confirm egg freezing services with them).

Typical Cost Estimates
Actual prices can vary by clinic, your health needs, medication protocol, and storage choices.
Approximate cost range:
Egg freezing cycle (procedure): ~R40 000–R80 000 + medication costs (often R10 000–R20 000)
First year storage: ~R5 000–R8 000
Consultation & scans: ~R1 000–R5 000

Clinic-specific examples

- *Aevitas:* clinic quotes can vary; consultations and cycle plans are personalised - ask about exact fees on inquiry.
- *HART:* one report suggests around **R53 000** for egg freezing cycles.
- *Other services in Cape Town* list egg freezing starting from lower figures (e.g., R18 100) - ask clinics for up-to-date quotes including meds and storage.

Medication is a big part of total cost - always ask clinics if they include medication and scans in their quote.
Most **medical aids** in South Africa don't fully cover fertility preservation, so plan for out-of-pocket costs.

Tips When Contacting Clinics
When you reach out, you might ask:
Do you offer **egg freezing** (oocyte cryopreservation)?
What's **included in the price** (procedure, medication, tests, storage)?
Do you support **transgender patients** and can tailor care?
How many **cycles or eggs** are recommended for best results?
What are your **storage fees** and policies?

Monitoring and follow up

Your doctor will advise and guide you in monitoring your safety while you are on hormone therapy. Usually, this will involve regular check-ups and physical examinations, as well as certain blood tests.

At the outset, it is not uncommon for these evaluations to be done monthly, while you are still achieving the correct hormonal balance for you. Later, once you are stable on treatment, these intervals might be extended to six-monthly, or perhaps even annually. This schedule is different for every client.

Cessation of hormone therapy

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If you decide to stop your hormone therapy, **you should discuss this decision with your doctor.** It can be dangerous to abruptly withdraw hormone therapy without adequate medical supervision.

More information

Please remember that you can discuss any questions or concerns with your doctor at any time.
Information on self-injection technique can be found at:

Youtube:

1) **How to do a testosterone or oestrogen injection – LGBTQ Clinic @uihealthcare**

https://www.youtube.com/watch?v=e0wDPZ0D9Wk

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